

Memorandum

TO: Rwandan Ministry of Health

FROM: Yvanna Gong

RE: Increasing Uptake of Long-Acting Reversible Contraception (LARC) in Rwanda: Insights from Recent Findings
DATE: Dec 10, 2024

The purpose of this memo is to provide an expert briefing on the 2019 study titled "Community Health Worker Promotions Increase Uptake of Long-Acting Reversible Contraception in Rwanda" and align its findings with actionable recommendations for the Rwandan Ministry of Health. (Mazzei et al., 2019) and provides recommendations for advancing family planning initiatives in Rwanda. The study highlights the effectiveness of community health worker (CHW) engagement in promoting long-acting reversible contraception (LARC) methods, specifically hormonal implants and intrauterine devices (IUDs), in Kigali. These findings are relevant to addressing Rwanda's high fertility rates, which average four children per woman, and its unmet need for family planning, affecting nearly 20% of women.

Past efforts by the Rwandan Ministry of Health (MoH) have successfully integrated CHWs into family planning services, establishing a strong foundation for leveraging community resources to address public health challenges. including initiatives such as the 2010 policy endorsing injectable contraception delivery by CHWs and the 2012 family planning policy emphasizing their role. Building on this foundation, the integration of LARC-focused counseling and clinical capacity could significantly enhance reproductive health outcomes by meeting demand for longer-term contraception.

The study, conducted between August 2015 and September 2016, systematically evaluated the dual impact of CHW-led community promotions and concurrent nurse training programs on LARC uptake in a practical, community-based setting in eight government clinics in Kigali

(Mazzei et al., 2019). CHWs provided fertility goal-based family planning counseling, often engaging couples, and issued written referrals for LARC methods. Concurrently, clinic nurses received training in LARC insertion and removal to address barriers such as provider bias and lack of clinical capacity.

Key Findings

1. CHWs issued 7,712 referrals, leading to 6,072 family planning visits (79% referral uptake).
2. 95% of these visits resulted in LARC uptake, with hormonal implants being the predominant choice (79% of users).
3. Monthly implant insertions tripled from 109 to 309, while IUD insertions doubled from 29 to 61 after implementation.
4. LARC uptake was more likely among women referred as couples, those with more children, and women with a religious affiliation. Educational level influenced method choice, with IUDs preferred by women with secondary or higher education.

Limitations

While the findings are compelling, the study's reliance on programmatic data without randomization introduces potential bias. Variability among clinics and providers also highlights the need for scalable training and standardized protocols.

Interpretation and Implications

The study demonstrates that integrating CHW-led counseling with clinical capacity-building can significantly enhance LARC adoption (Mazzei et al., 2019). Further support for this

approach comes from other studies, such as one conducted in the United States demonstrating that enhanced training and supply chain improvements also lead to increased uptake of LARC methods (Buckel et al., 2019). This approach aligns with Rwanda's strategic priorities to improve maternal and child health through equitable access to family planning. The following implications merit consideration:

1. Expanding CHW training nationwide to include LARC-focused counseling could bridge gaps in community-level family planning education and demand generation.
2. Emphasizing couple-based counseling may address sociocultural barriers, enhance male partner involvement, and align family planning goals within households.
3. Ensuring consistent nurse training and on-site LARC availability is critical to meeting increased demand. This includes addressing logistical challenges such as supply chain management and clinic workflow.
4. Robust data collection systems are essential for evaluating program effectiveness and guiding iterative improvements. Leveraging Rwanda's strong health information systems can support this effort.

Recommendations and Next Steps

To build on these findings, the Ministry of Health should consider the following actions, which are directly informed by the study's conclusions and tailored to Rwanda's healthcare priorities:

1. Scale up training for CHWs and clinic staff on LARC methods, emphasizing practical skills and addressing misconceptions.

2. Develop campaigns that normalize LARC use, leveraging CHWs' community trust and focusing on both individual and couple-based counseling.
3. Ensure uninterrupted availability of LARC supplies through strategic partnerships with commodity providers.
4. Implement pilot programs in additional regions to validate the Kigali findings before national rollout.
5. Advocate for policies that incentivize CHWs and clinic staff for LARC service delivery, building on Rwanda's performance-based financing framework.

Conclusion

The integration of CHW-led LARC promotion and nurse training has demonstrated substantial potential to enhance family planning services in Rwanda by linking actionable recommendations directly to data-driven findings, ensuring that policy decisions are grounded in robust evidence. (Mazzei et al., 2019). By addressing structural barriers and leveraging existing community health infrastructure, the Ministry of Health can achieve transformative improvements in reproductive health outcomes, contributing to national goals of reducing fertility rates and empowering families with effective contraceptive options.

References

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